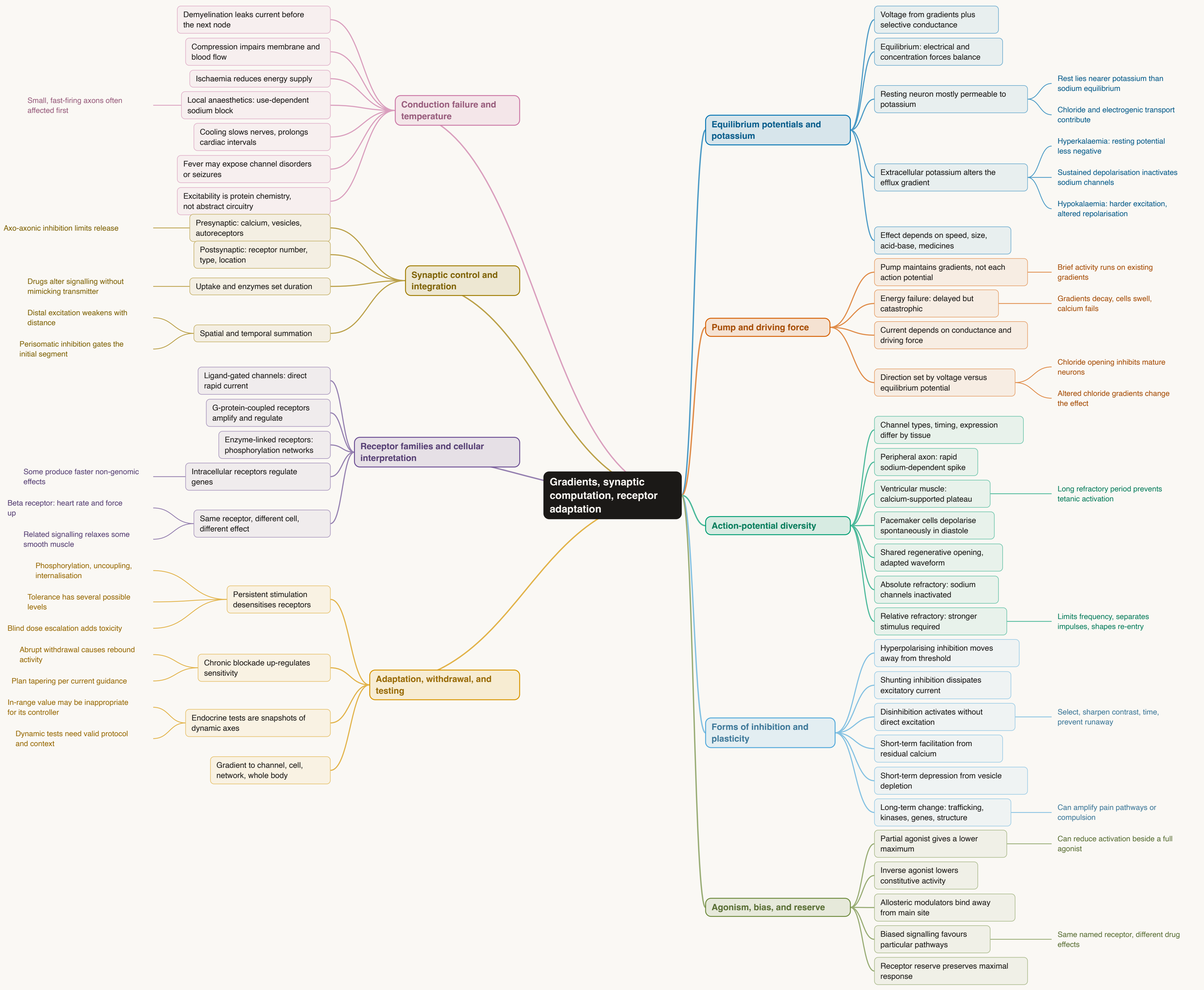




Small, fast-firing axons often affected first

Axo-axonic inhibition limits release

Drugs alter signalling without mimicking transmitter

Distal excitation weakens with distance

Perisomatic inhibition gates the initial segment

Some produce faster non-genomic effects

Beta receptor: heart rate and force up

Related signalling relaxes some smooth muscle

Phosphorylation, uncoupling, internalisation

Tolerance has several possible levels

Blind dose escalation adds toxicity

Abrupt withdrawal causes rebound activity

Plan tapering per current guidance

In-range value may be inappropriate for its controller

Dynamic tests need valid protocol and context

Rest lies nearer potassium than sodium equilibrium

Chloride and electrogenic transport contribute

Hyperkalaemia: resting potential less negative

Sustained depolarisation inactivates sodium channels

Hypokalaemia: harder excitation, altered repolarisation

Brief activity runs on existing gradients

Gradients decay, cells swell, calcium fails

Chloride opening inhibits mature neurons

Altered chloride gradients change the effect

Long refractory period prevents tetanic activation

Limits frequency, separates impulses, shapes re-entry

Select, sharpen contrast, time, prevent runaway

Can amplify pain pathways or compulsion

Can reduce activation beside a full agonist

Same named receptor, different drug effects